

ACGME Family Medicine Procedural Requirements & Documentation Reference

UC Riverside (UCR) Resident Edition

Contents

- TL;DR
- Key Findings
 - 1. What the current ACGME requirements actually say
 - 2. ABFM required procedures (competence required by graduation)
 - 3. STFM core procedure list (the most widely cited “should perform by graduation” list)
 - 4. UCR-specific findings
- Details
 - A. Benchmark minimum numbers (labeled as benchmarks, NOT ACGME mandates)
 - B. STFM Core Procedures (Category A — “all programs must train”)
 - C. Core elements of ANY procedure note
 - D. Copy-pasteable procedure-note templates
 - E. What to log in the procedure log (ACGME/program fields)
 - F. Procedure-tracking spreadsheet schema (deliverable #2)
- Recommendations
- Caveats

TL;DR

- **ACGME no longer mandates national minimum procedure NUMBERS in family medicine.** Since the July 1, 2023 Program Requirements, procedural adequacy is judged by competency (program director + Clinical Competency Committee), not counts. The ABFM's current guidance lists **13 required procedures** every FM resident must be judged *competent* in by graduation and states verbatim: *"At this time, programs are not required to submit procedure counts to ABFM."*
- **Use benchmark numbers, not mandates.** Any specific number (e.g., 40 vaginal deliveries, 5 skin biopsies) is a *historical* ACGME figure, an STFM/AAFP curricular recommendation, or a hospital-credentialing convention — **not** a current ACGME rule. UCR does not publish its own numeric procedure minimums; default to the ABFM required list + STFM core list + the benchmark table below, and confirm UCR's internal list with your coordinator.
- **This report delivers two things:** (1) a copy-pasteable procedure-note reference for the major FM procedures, and (2) a procedure-tracking spreadsheet schema with the exact fields ACGME/New Innovations/MedHub expect.

Key Findings

1. What the current ACGME requirements actually say

- The governing document is the **ACGME Program Requirements for Graduate Medical Education in Family Medicine** (effective July 1, 2023; reformatted 2025 and 2026 editions on [acgme.org](https://www.acgme.org)). It requires residents to "receive training to perform those clinical procedures" needed in family medicine but sets **no national minimum numbers**.
- The **2014** revision removed the old volume-based maternity requirement (40 deliveries; ≥ 30 vaginal; ≥ 10 continuity). As AAFP summarizes (Loeven, *Am Fam Physician*, May 2022): *"The Accreditation Council for Graduate Medical Education's requirement that physicians perform a minimum number of deliveries during residency training was removed in 2014"* (the same article notes only 6.7% of family physicians now perform deliveries).
- The **2023** revision replaced the "1,650 continuity visits" rule with panel-size and competency metrics; programs may still use 1,650 as *one* internal measure of competence, but it is no longer cited.
- **Competency-Based Board Eligibility (CBBE):** The ABFM is phasing in program-director attestation of resident competence across core outcomes. Importantly, per the ABFM's December 2025 guidance, **procedural attestation has been postponed**: *"ABFM has postponed requirement for program directors to attest to procedural competence as part of a resident's eligibility for ABFM Board Certification until June 2027. There will be an optional attestation available in 2026."* Fourteen core outcomes are required for June 2026 eligibility; procedures become a required attestation in June 2027.

2. ABFM required procedures (competence required by graduation)

Per the ABFM's Core Competencies and Procedures guidance, *"The following is a list of required procedures in which all Family Medicine residents must demonstrate competence by the time of graduation"* — **13 items** (12 procedural skills plus a life-support certification bundle): - Biopsy, skin (excisional, punch, or shave) - Bracing/splinting of upper extremity or ankle - Destruction of skin lesions; acrochordon (skin tag) removal - EKG interpretation - I&D superficial abscess - Interpretation of basic x-rays (chest, KUB, spine, extremities) - Joint injection/aspiration of large joints (knee, shoulder) - LARC: IUD insertion/removal **OR** implant insertion/removal (conscientious-objection option at verification) - Pap smear sampling and interpretation - Simple laceration repair with sutures; suture/staple removal - Toenail procedures (ingrown-nail excision; onychomycosis management) - Trigger point injections - Initial certification in **ACLS, ALSO, and NRP/NALS**

POCUS becomes required in 2027: *"POCUS will be a required procedure in 2027... The POCUS training requirement is currently pending the outcome of STFM's Project FOCUS."*

The ABFM does **not** require attestation for a long "additional/optional" list — colposcopy, EGD, colonoscopy, vasectomy, circumcision, central lines, LP, paracentesis, thoracentesis, chest tube, intubation, treadmill stress testing, casting, LEEP, D&C, arterial lines, PICC lines, and more — but encourages programs to train these based on mission and community need, and to publicly disclose which they teach to competence.

3. STFM core procedure list (the most widely cited "should perform by graduation" list)

From Nothnagle et al., *Fam Med* 2008;40(4):248-252 and Kelly et al., *Fam Med* 2009;41(6):398-404 (STFM Group on Hospital Medicine and Procedural Training). **Category A = all programs must train.** Full cluster breakdown in Details §B. This is the foundation later expanded by the 2014 CAFM consensus statement.

4. UCR-specific findings

- The **UCR School of Medicine Family Medicine Residency Program** (somfm.ucr.edu) is a three-year program that **offers eight positions per year** (per UCR Health), primary participating site **Dignity Health St. Bernardine Medical Center** (San Bernardino); NRMP code **1522120C2**; continuity care via UCR Health.
- A **separate, distinct** program exists: the **RUHS/UCR Family Medicine Residency** in Moreno Valley (ACGME 1200521421), which describes itself as a **"12-12-12"** program with continuity clinic at Moreno Valley Community Health Center (an FQHC). Confirm which program you are in — requirements and logging differ.
- **UCR's public curriculum shows dedicated procedural exposure:** a PGY-2 block **"Procedures" rotation at RUHS-Rubidoux (4 weeks)**; PGY-1 Orthopedics + PGY-2 Sports Medicine (casting/splinting/joint injections); PGY-1/PGY-2 Obstetrics blocks (8 weeks total at SBMC L&D); and a **PGY-3 Gynecology rotation** covering colposcopy, endometrial biopsy, and IUD/Nexplanon.
- UCR does **not** publish a numeric required-procedure list or log minimums publicly. Its Program Objectives page lists only competency goals ("Prescribe and perform procedures"). **Default**

to ABFM + STFM + the benchmark table, and confirm UCR’s exact list with your program coordinator/DIO.

Details

A. Benchmark minimum numbers (labeled as benchmarks, NOT ACGME mandates)

The most-cited numeric table for FM personal targets is the “Procedure List for Family Medicine Residency” (Time of Care), which reflects typical hospital-credentialing supervised-procedure counts. **These are credentialing/benchmark numbers — useful as stretch targets, not requirements.**

Procedure	Benchmark #	Source type
Abscess I&D	3	Credentialing benchmark
Punch skin biopsy	3	Credentialing benchmark
Shave biopsy	5	Credentialing benchmark
Excision, lesion	5	Credentialing benchmark
Skin tags, excise	3	Credentialing benchmark
Cryotherapy	5	Credentialing benchmark
Laceration repair	5	Credentialing benchmark
Ingrown toenail excision	5	Credentialing benchmark
Joint injection	5	Credentialing benchmark
Joint aspiration	5	Credentialing benchmark
Trigger point injection	3	Credentialing benchmark
Cast/splint – arm	3	Credentialing benchmark
Cast/splint – leg	3	Credentialing benchmark
Reduction of acute joint dislocation	5	Credentialing benchmark
Vaginal delivery	40	Historical ACGME (pre-2014)
Colposcopy	10	Credentialing benchmark
Endometrial biopsy	5	Credentialing benchmark
Cervical biopsy	5	Credentialing benchmark
IUD insertion	5	Credentialing benchmark
IUD removal	3	Credentialing benchmark
Pap smear	5	Credentialing benchmark
Circumcision	10	Credentialing benchmark

Procedure	Benchmark #	Source type
Vasectomy	5	Credentialing benchmark
EKG interpretation	5	Credentialing benchmark
Chest x-ray interpretation	10	Credentialing benchmark
Endotracheal intubation	10	Credentialing benchmark
Central venous line	15	Credentialing benchmark
Lumbar puncture	5	Credentialing benchmark
Paracentesis	5	Credentialing benchmark
Thoracentesis	5	Credentialing benchmark
Chest tube insertion	10	Credentialing benchmark

Maternity context: Prior to 2014, ACGME required 40 deliveries (≥ 30 vaginal, ≥ 10 continuity) — removed in 2014. For *practice* obstetric credentialing, the ABPS Board of Certification in Family Medicine Obstetrics (BCFMO, updated Jan 2023) requires applicants to “*Demonstrate evidence of ongoing obstetrical practice by having completed 60 vaginal deliveries, 15 of which must have been in the last 2 years.*” Some programs still set internal targets (e.g., Kaiser Napa/Solano lists a baseline of 40 deliveries + 10 continuity deliveries).

B. STFM Core Procedures (Category A — “all programs must train”)

Category levels: **A0** = expected from med school/normal residency (no documentation of numbers needed); **A1** = must perform independently by graduation; **A2** = must have exposure/opportunity to be trained to independence.

- **Skin:** (A1) punch/excisional/incisional biopsy, cryosurgery, wart/nail/foreign-body removal, I&D abscess, simple laceration repair, electrosurgery; (A0) corn/callus removal, drain subungual hematoma, staples, KOH prep, tissue-glue closure.
- **Maternity:** (A1) spontaneous vaginal delivery incl. fetal monitoring, scalp electrode, IUPC/ amnioinfusion, amniotomy, induction/augmentation, 1st/2nd-degree laceration repair; (A2) vacuum-assisted delivery, 3rd/4th-degree repair, manual placenta extraction.
- **Women’s health:** (A1) Pap, vulvar biopsy, Bartholin’s cyst management, cervical polyp removal, endometrial biopsy, IUD insertion/removal, breast FNA, pessary fitting, paracervical block, cervical dilation; (A2) colposcopy, cervical cryotherapy, uterine aspiration/D&C; (A0) wet mount/KOH, diaphragm fitting.
- **Life support:** (A0) EKG performance/interpretation; (A1) ACLS, NRP, PALS, ALSO, ATLS.
- **MSK:** (A1) simple fracture reduction, upper/lower-extremity splints, large-joint/bursa/ ganglion/trigger-point injection-aspiration, nursemaid’s-elbow reduction; (A2) upper/lower-extremity casts, shoulder-dislocation reduction.
- **Pulmonary:** (A1) handheld spirometry.
- **Ultrasound:** (A1) basic OB US (AFI, presentation, placenta); (A2) US-guided central access/ paracentesis/thoracentesis, advanced OB US (dating, anatomic survey).

- **Urgent/Hospital:** (A1) eye fluorescein/FB removal, anterior nasal packing, lumbar puncture, FNA, frenulotomy, slit-lamp, endotracheal intubation; (A2) ventilator management, thoracentesis, paracentesis, arterial line, central venous catheter, venous cutdown, pediatric vascular access (peripheral/IO/umbilical); (A0) ENT/ring/fishhook FB removal, phlebotomy, peripheral IV.
- **GI/Colorectal:** (A1) anoscopy, thrombosed-hemorrhoid excision, perirectal-abscess I&D, perianal skin-tag removal; (A2) flexible sigmoidoscopy/colonoscopy; (A0) NG/feeding tube, disimpaction, DRE.
- **GU:** (A1) newborn circumcision; (A2) vasectomy, suprapubic aspiration; (A0) urine microscopy, bladder catheterization.
- **Anesthesia:** (A0) topical/local/field block; (A1) digital block, peripheral nerve block; (A2) conscious sedation.

STFM Advanced Procedures (Category B/C — 36 procedures) include: contraceptive-implant insertion/removal, LEEP, tubal ligation, D&E, cesarean delivery, forceps delivery, external cephalic version, amniocentesis, cervical cerclage, EGD, exercise stress test, chest-tube insertion, cardioversion, nasolaryngoscopy, peritonsillar-abscess I&D, Swan-Ganz, tooth extraction, botulinum toxin, non-neonatal circumcision, bronchoscopy, myringotomy (PE) tubes, appendectomy, epidural/intrathecal anesthesia.

C. Core elements of ANY procedure note

Date/time; **indication / pre-procedure diagnosis; informed consent** (risks/benefits/alternatives discussed, questions answered, verbal/written consent obtained); **time-out** (correct patient/site/procedure); provider + supervising physician; anesthesia/medications (agent, concentration, volume, and **lot #/expiration for implants**); sterile prep/technique; **step-by-step description**; findings; **specimens sent** (and destination); estimated blood loss; complications; patient tolerance; post-procedure condition; disposition/follow-up + return precautions; CPT/ICD-10.

D. Copy-pasteable procedure-note templates

*(Adapt every field to the actual patient. **Never clone notes** — cloning is unethical, unsafe, and potentially fraudulent. Templates synthesized from Ventura County Medical Center FM Residency autotexts, Time of Care, Reproductive Health Access Project, WikEM, and AAFP sources.)*

1) SKIN BIOPSY (shave/punch/excisional)

PROCEDURE: Skin biopsy [shave / punch (___mm) / excisional]
 INDICATION: _____ (e.g., suspicious pigmented lesion, r/o malignancy)
 CONSENT: Risks (bleeding, infection, scarring, incomplete removal, recurrence), benefits, alternatives discussed; questions answered; verbal/written consent obtained.
 TIME-OUT: Correct patient, site (___), procedure verified.
 SITE/PREP: Lesion at ____; prepped with [alcohol/chlorhexidine]; anesthetized with ___ mL 1% lidocaine [with/without epi].
 TECHNIQUE: Lesion removed by [dermablade shave / ___mm punch rotated to subcutis / elliptical excision]. Hemostasis with [aluminum chloride / Monsel's / electrocautery / pressure].
 CLOSURE: [None / ___ interrupted ___-0 nylon sutures].
 SPECIMEN: Placed in formalin, sent to pathology.
 EBL: minimal. COMPLICATIONS: none. TOLERANCE: well.
 POST-OP/FOLLOW-UP: Wound care reviewed; suture removal in ___ days; will call with path; return precautions given.
 CPT: 11102-11107 (biopsy) / 11400-series (excision)

2) CRYOTHERAPY (liquid nitrogen)

PROCEDURE: Cryotherapy, liquid nitrogen (LN2)
 INDICATION: _____ (e.g., verruca, actinic keratosis, seborrheic keratosis)
 CONSENT: Risks (pain, blistering, hypo/hyperpigmentation, scarring, recurrence) discussed; consent obtained.
 TECHNIQUE: LN2 applied via [cryospray/cotton-tip] to lesion(s) at ____ with 1-2 mm margin; ___ freeze-thaw cycle(s) x ___ seconds each. [Shorthand: LN2 ___FTC ___s]
 COMPLICATIONS: none. TOLERANCE: well.
 FOLLOW-UP: Blistering expected; wound care reviewed; return/re-treat in ___ weeks PRN.
 CPT: 17110/17111 (benign warts) or 17000-series (AK)

3) INCISION & DRAINAGE OF ABSCESS

PROCEDURE: Incision and drainage, [simple 10060 / complex 10061]
 INDICATION: Fluctuant abscess at ____ (size ___ cm).
 CONSENT/TIME-OUT: Completed.
 PREP/ANESTHESIA: Sterile prep; field block/local with ___ mL 1% lidocaine [with/without epi].
 TECHNIQUE: #11 blade linear incision along skin tension lines; ___ mL purulent material expressed; loculations disrupted with hemostat; irrigated with normal saline.
 CULTURE: [sent / not sent]. PACKING: [iodoform gauze placed / not packed].
 EBL: minimal. COMPLICATIONS: none. TOLERANCE: well.
 FOLLOW-UP: Wound check/packing removal in 2-3 days; return precautions (fever, spreading erythema); +/- antibiotics.
 CPT: 10060 / 10061

4) LACERATION REPAIR

PROCEDURE: Laceration repair, [simple / intermediate], length ___ cm at ____
 INDICATION: Traumatic laceration.
 NEUROVASCULAR EXAM: Intact distal to injury; no tendon/foreign body on exploration.
 CONSENT/TIME-OUT: Completed.
 ANESTHESIA: ___ mL 1% lidocaine [with/without epi] local infiltration.
 PREP: Irrigated with ___ mL normal saline; sterile drape.
 CLOSURE: ___ [simple interrupted] sutures of ___-0 [nylon/Vicryl]; [+/- deep dermal layer].
 EBL: minimal. COMPLICATIONS: none. TOLERANCE: well.
 FOLLOW-UP: Wound care; suture removal in ___ days (face 5, scalp/trunk 7-10, extremity 10-14); tetanus updated PRN; return precautions.
 CPT: 12001-13160 by site/length/complexity

5) JOINT INJECTION / ASPIRATION (arthrocentesis)

PROCEDURE: [Injection / aspiration] of __ joint (e.g., knee), [with/without] US guidance
INDICATION: _____ (e.g., osteoarthritis, effusion r/o septic).
CONSENT/TIME-OUT: Completed.
PREP/ANESTHESIA: Sterile prep; __ mL 1% lidocaine skin wheal.
TECHNIQUE: __ gauge needle via [superolateral/anteromedial] approach into joint space.
ASPIRATION: __ mL [color] synovial fluid; sent for [cell count, crystals, culture, gram stain].
INJECTION: __ mg [triamcinolone/methylprednisolone] + __ mL lidocaine.
FINDINGS: Immediate relief noted [y/n]. EBL: minimal. COMPLICATIONS: none. TOLERANCE: well.
FOLLOW-UP: Post-injection flare precautions; effect in __ days.
CPT: 20610/20611 (major joint +/- US), 20605/20606 (intermediate), 20600/20604 (small)

6) TRIGGER POINT INJECTION

PROCEDURE: Trigger point injection, __ site(s) (muscle: ____)
INDICATION: Myofascial pain. CONSENT: Obtained.
TECHNIQUE: Sterile prep; __ gauge needle into palpable trigger point(s); __ mL [1% lidocaine +/- __ mg corticosteroid] injected per site.
COMPLICATIONS: none. TOLERANCE: well. FOLLOW-UP: PRN.
CPT: 20552 (1-2 muscles) / 20553 (≥3 muscles)

7) TOENAIL REMOVAL / MATRIXECTOMY (ingrown nail)

PROCEDURE: [Partial/complete] toenail avulsion [+/- chemical matrixectomy], __ toe
INDICATION: Ingrown toenail / onychocryptosis. CONSENT/TIME-OUT: Completed.
ANESTHESIA: Digital block with __ mL 1% lidocaine [without epi].
TECHNIQUE: Nail border/plate elevated and removed; [phenol applied x __ for matrixectomy; neutralized with alcohol].
HEMOSTASIS: [tourniquet released; pressure]. COMPLICATIONS: none. TOLERANCE: well.
FOLLOW-UP: Wound care; soaks; return precautions.
CPT: 11730/11732 (avulsion), 11750 (excision w/ matrixectomy)

8) IUD INSERTION

PROCEDURE: IUD insertion – type: [Mirena/Kyleena/Liletta/Skyla/Paragard], Lot#__ Exp__
INDICATION: Contraception (Z30.014). Pregnancy/STI excluded.
CONSENT: Risks (expulsion, perforation, infection, malposition, failure/ectopic) discussed; consent signed.
TIME-OUT: Completed.
TECHNIQUE: Bimanual exam – uterus [ante/retroverted]. Speculum placed; cervix visualized and cleansed. Tenaculum on anterior lip; uterus sounded to __ cm. IUD loaded and inserted to fundus per manufacturer technique; strings trimmed to __ cm.
EBL: minimal. COMPLICATIONS: none. TOLERANCE: well.
FOLLOW-UP: Expulsion/string precautions; string check; return precautions.
CPT: 58300; supply J7297-J7301 by device

9) NEXPLANON / CONTRACEPTIVE IMPLANT INSERTION

PROCEDURE: Etonogestrel implant (Nexplanon) insertion – Lot#__ Exp__
 INDICATION: Contraception (Z30.017).
 CONSENT: Risks/benefits/alternatives discussed; consent obtained.
 SITE: Non-dominant arm; insertion site marked 8-10 cm proximal to medial epicondyle over triceps; guiding mark placed.
 PREP/ANESTHESIA: Sterile prep [betadine/chlorhexidine]; 2 mL 1% lidocaine subdermal along tunnel.
 TECHNIQUE: Applicator inserted subdermally; implant delivered; needle withdrawn. Implant PALPATED by provider AND patient to confirm placement.
 EBL: <0.5 mL. COMPLICATIONS: none. TOLERANCE: well.
 DRESSING: Pressure dressing x24h; bandage 3-5 days. FOLLOW-UP: 3-yr replacement; return precautions.
 CPT: 11981 (insertion); J7307 (supply)

10) ENDOMETRIAL BIOPSY (Pipelle)

PROCEDURE: Endometrial biopsy (Pipelle)
 INDICATION: _____ (e.g., abnormal/postmenopausal uterine bleeding).
 CONSENT/TIME-OUT: Completed. Pregnancy excluded.
 TECHNIQUE: Bimanual exam – uterus position noted. Speculum placed; cervix cleansed [+/- tenaculum, +/- paracervical block]. Pipelle advanced to fundus (depth __ cm); suction created; tissue obtained with 360° rotation x __ passes.
 SPECIMEN: __ cc tissue to formalin, sent to pathology.
 EBL: minimal. COMPLICATIONS: none. TOLERANCE: well.
 FOLLOW-UP: Cramping/spotting expected; will call with path; return precautions.
 CPT: 58100 (without dilation) / 58120 (with D&C)

11) COLPOSCOPY

PROCEDURE: Colposcopy [+/- cervical biopsy, +/- ECC]
 INDICATION: Abnormal Pap / positive HPV. CONSENT: Obtained.
 TECHNIQUE: Speculum placed; cervix visualized. 3-5% acetic acid applied [+/- Lugol's]. Squamocolumnar junction [visualized/not]. Acetowhite/vascular changes at _____. Biopsies from ____ o'clock positions; ECC performed [y/n]. Monsel's for hemostasis.
 SPECIMENS: Labeled by site, to pathology.
 COMPLICATIONS: none. TOLERANCE: well. FOLLOW-UP: Path-driven; return precautions.
 CPT: 57452 (colpo alone), 57454 (w/ biopsy + ECC), 57455/57456, 57460 (LEEP)

12) SKIN TAG / ACROCHORDON REMOVAL

PROCEDURE: Removal of skin tag(s), __ lesion(s) at ____
 INDICATION: Symptomatic/irritated acrochordon. CONSENT: Obtained.
 TECHNIQUE: [Snip excision with iris scissors / electrodesiccation / cryotherapy] after [+/- local anesthesia]. Hemostasis with [aluminum chloride/pressure].
 COMPLICATIONS: none. TOLERANCE: well. FOLLOW-UP: PRN.
 CPT: 11200 (up to 15) / 11201 (each additional 10)

13) LUMBAR PUNCTURE (hospital)

PROCEDURE: Lumbar puncture
 INDICATION: _____ CONSENT/TIME-OUT: Completed.
 POSITION: [lateral decubitus/sitting]. PREP: Sterile; 1% lidocaine local.
 TECHNIQUE: __ gauge [Quincke/attraumatic] needle at [L3-4/L4-5]; opening pressure __ cm H2O; __ mL clear CSF collected in __ tubes.
 STUDIES: cell count, glucose, protein, culture, [other].
 COMPLICATIONS: none. TOLERANCE: well. FOLLOW-UP: post-LP precautions.
 CPT: 62270

14) PARACENTESIS / THORACENTESIS / CENTRAL LINE / ARTERIAL LINE / INTUBATION (skeleton) Use the same structure: indication → consent/time-out → sterile prep + anesthesia → **ultrasound guidance noted** → technique (site, needle/catheter gauge, Seldinger for lines) → volume/fluid removed + studies sent → **confirmation** (blood return/flow; **post-procedure CXR for central line & intubation** confirming position and no pneumothorax) → EBL/complications/tolerance → follow-up. (Patterned on WikEM/VCMC procedure notes.)

E. What to log in the procedure log (ACGME/program fields)

FM programs generally do **not** use the ACGME ADS Case Log system (unlike surgical specialties); they use **New Innovations** or **MedHub**, which feed the program's competency/privileging tracking. Standard log fields: - **Date** of procedure - **Procedure name / CPT code** - **Patient identifier** (MRN/age/sex per program privacy policy) - **Role** — the single most important field: *Observed / Assisted / Performed with direct supervision / Performed with indirect supervision / Performed independently / Supervised (as senior)*. Only roles designated to "count" advance your privileging tally. - **Supervisor / attending name** (often requires supervisor confirmation sign-off) - **Setting / location** (clinic, L&D, ED, inpatient) - **Diagnosis / indication** - **Outcome / complications**; optional competency grade/comment from supervisor

New Innovations supports **automatic privileging** (hitting a target number triggers "conditional independence") and **ACGME import**; MedHub offers EHR-integrated auto-logging (a UC San Diego study found EHR-to-MedHub API auto-logging produced a **78% increase in captured procedures at 99.5% accuracy** over one year).

F. Procedure-tracking spreadsheet schema (deliverable #2)

Log tab — one row per procedure:

- Date
- Procedure
- CPT
- Cluster (Skin/WH/MSK/OB/Hospital/GI/GU)
- Patient MRN/ID
- Age
- Sex
- Indication
- Role (Observed/Assist/Perf-Direct/Perf-Indirect/Independent/Supervised)
- Supervising Attending
- Setting/Site
- Outcome/Complication
- Supervisor Confirmed (Y/N)
- Notes

Dashboard tab — one row per procedure type:

- Procedure
- ABFM-Required? (Y/N)

- Personal Target (benchmark)
- Number Performed
- Number Remaining
- Competency Attained (Y/N)
- Last Performed Date

Pre-load the dashboard with the **13 ABFM-required procedures** (mark Required = Y) and the **STFM Category-A core list** so gaps are visible at a glance. Keep the personal-target column populated from the benchmark table (§A) as stretch goals.

Recommendations

1. **Month 1 (PGY-1):** Confirm with your UCR coordinator/DIO: (a) which logging system UCR uses (New Innovations vs MedHub), (b) UCR's own required-procedure list and any internal numeric targets, and (c) whether supervisor confirmation is mandatory. These are not published publicly, so ask early.
2. **Build the tracking spreadsheet now** (schema in §F), pre-loaded with the 13 ABFM-required procedures as non-negotiable graduation targets and the §A benchmarks as stretch goals. **Log the same day you perform a procedure** — deferred logging is the #1 cause of undercounting, especially for senior residents.
3. **Prioritize the 13 ABFM-required procedures** for competency sign-off; procedural attestation becomes required for board eligibility in **June 2027** (optional in 2026), so aim to have all 13 documented to competence well before your PGY-3 spring. Front-load skin/MSK during PGY-1 Ortho and PGY-2 Sports Med + the PGY-2 Procedures rotation at Rubidoux; women's-health procedures during the PGY-3 Gynecology block; deliveries during the PGY-1/PGY-2 OB blocks.
4. **Deploy the §D templates as EHR dot-phrases/auto-text**, but individualize every field. Confirm each note captures consent, time-out, supervising physician, specimen disposition, and follow-up — the elements auditors and the CCC scrutinize.
5. **Benchmarks that change the plan:** If you intend to practice **obstetrics**, aim far above residency exposure (practice credentialing/ABPS FM-OB wants ≥ 60 vaginal deliveries). If you want **colposcopy, endoscopy, or vasectomy** privileges, treat these as STFM "advanced" (Category B/C) procedures needing focused elective time and dedicated logs — ABFM does not require them.
6. **Start logging POCUS now** — it becomes ABFM-required in 2027. Track ultrasound-guided procedures and dedicated diagnostic scans separately in your spreadsheet.

Caveats

- **No current ACGME national minimum numbers exist for FM procedures.** Every number in §A is a benchmark/historical/credentialing figure, not an ACGME mandate. Your program's own thresholds govern.
- **UCR-specific numeric requirements could not be found on public sources;** UCR findings reflect its public curriculum, rotations, and objectives pages only. Two related-but-

distinct programs exist (UCR SOM / St. Bernardine, San Bernardino; and RUHS/UCR, Moreno Valley) — the “12-12-12” descriptor and continuity clinic differ between them. Verify which program you’re in.

- **The Time of Care benchmark table is undated/secondary** and reflects generic hospital-credentialing conventions, not an official UCR or ACGME source — use it only as a rough personal-target guide.
- **ABFM procedural attestation is postponed to June 2027** (optional in 2026); fourteen non-procedural core outcomes are required for June 2026 eligibility. Timelines are set by the ABFM’s December 2025 guidance and could be revised — reconfirm annually.
- Templates provide documentation *structure*; clinical technique details (needle gauge, drug doses) must follow your attending’s guidance and current evidence. **Do not clone notes.**
- The STFM lists (2008/2009) predate the 2014 CAFM consensus statement; some literature conflates them. For board eligibility, the ABFM required list is the current authority.